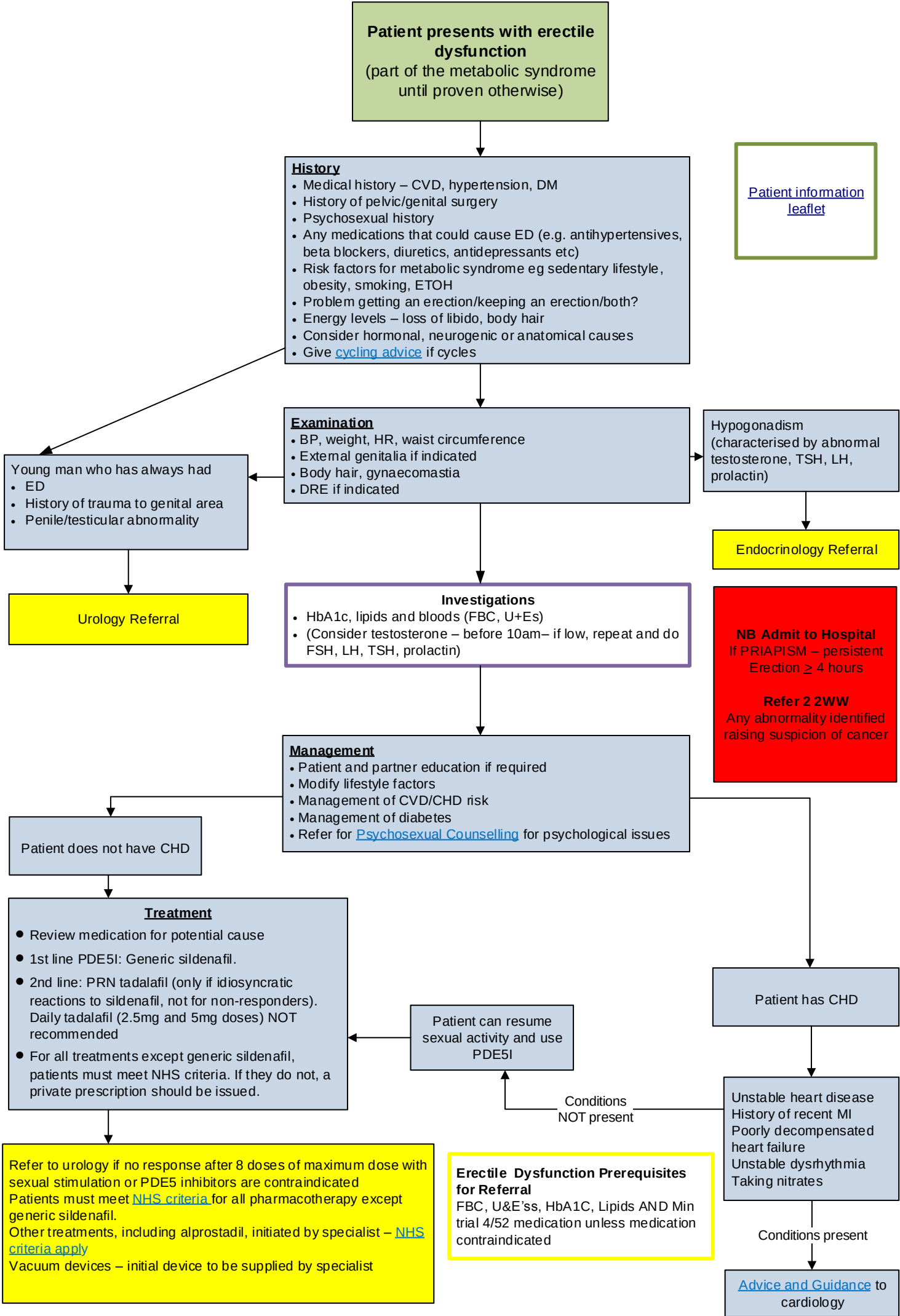


Camden Erectile Dysfunction pathway

This pathway has been developed from published guidance, in collaboration with local urologists.

This guidance is to assist GPs in decision making and is not intended to replace clinical judgment.



Comments & enquiries relating to medication:
CCCG Medicines Management Team mmt.camdenccg@nhs.net
Refer to current BNF or SPC for full medicines information

Pathway created by NCL led by Camden CCG Clinical Cabinet + GB July 2016

Review due – March 2020

Clinical Contact for this pathway for queries:
camden.pathways@nhs.net

Cycling Advice

Nearly every cyclist who has erectile dysfunction as a result of cycling will have experienced prior symptoms of pain, numbness or tingling in the perineum due to pressure on the perineum affecting the underlying blood vessels and nerves.

- If cycling >3hours/week try period of time without cycling
- Ride in the correct position ie changing position frequently during ride and sitting more upright rather than leaning forward over handlebars to reduce pressure on the perineum
- Use a properly fitted seat, level or angled slightly downwards – never tilt saddle upwards.
- Use a seat with a cut out to reduce pressure on the perineum
- Wear padded cycle shorts

Examples of dug causing ED

Antihypertensives – thiazides, beta blockers

Psychotropics drugs – phenothiazine's, tricyclics, MAOI's SSRIs, Lithium

Hormone modifying drugs – cyproterone acetate, oestrogens, 5-alpha reductase inhibitors, corticosteroids, progesterone

Recreational drugs – alcohol, tobacco, cannabis

Others – digoxin, ranitidine, carbamazepine, spironolactone

NHS Criteria

Laid down by department of Health as described in the Health Service Circular 1999/148 (amended 2009) .

NHS criteria for erectile dysfunction pharmacotherapy (excluding generic sildenafil which can be prescribed on the NHS to any patient with erectile dysfunction regardless of cause, where this is clinically appropriate).

- Patients suffering from any of the following:

Diabetes

Multiple Sclerosis

Parkinson's Disease

Poliomyelitis

Prostate cancer

Severe pelvis injury

Single gene neurological disease

Spina Bifida

Spinal cord injury

- Patients receiving treatment for renal failure by dialysis
- Patients who have had thee following surgery

Prostatectomy

Radical pelvic surgery

Kidney Transplant
- Patients who were receiving NHS prescriptions for alprostadil, apomorphine, moxislyte, slidenafil or thymoxamine HCL on 14th September 1998
- [None – UK Nationals \(EEA nationals\) may have the right to NHS provision of erectile dysfunction therapy. Contact the medicines management team for further guidance.](#)

PDE5 inhibitors are contraindicated in patients:

- Receiving nitrates in any form
- In whom vasodilation or sexual activity is not recommended including those with unstable heart disease, recent stroke, unstable angina, a history of recent MI – within 6/12, poorly compensated HF, or unstable dysrhythmia – refer to [CVD risk](#) for more info
- Who have lost sight in one eye due to non-arteritic anterior ischaemic optic neurology (NAION)
- With hypotension (systolic<90mmHg),
- *Sildenafil* – CI in severe hepatic impairment or hereditary retinal degenerative disorders such as retinitis pigpmentosa
- *Tadalafil* – CI - men with NYHA class 2 or greater heart failure in last 6/12, uncontrolled arrhythmias, or uncontrolled hypertension

- **Caution in patients** with:

Cardiovascular disease

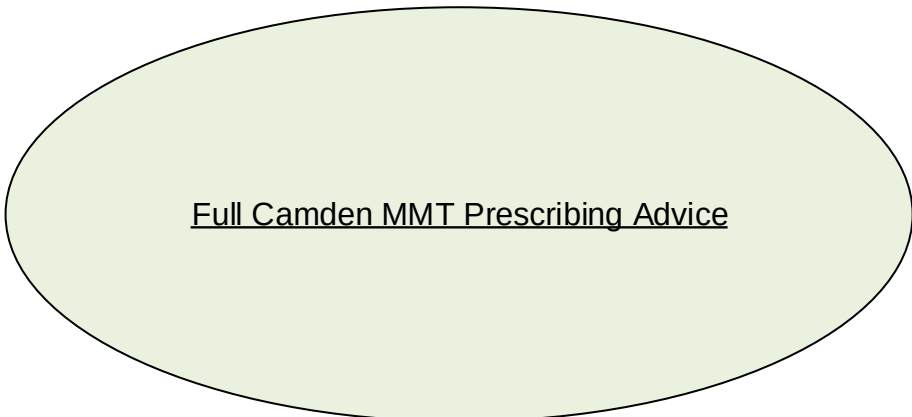
Left ventricular outflow obstruction

Anatomical deformation penis

A predisposition to priapism eg in sickle cell disease, multiple myeloma or leukaemia

Side Effects PDE5 inhibitors include

- Headache
- Nasal Congestion
- Flushing
- Dyspepsia
- Myalgia



Non-UK National (EEA and non-EEA nationals)

In England Alprostadil, avanafil, tadalafil, Viagra, are permitted to be prescribed for the management of erectile dysfunction for the following patients:

A)A man who is a national of an A state who is entitled to treatment by virtue of Article 7 (2) of Council Regulations 1612/68© as extended by the EEA agreement by virtue of any other enforceable Community right now has erectile dysfunction and was on 14 September 1998 receiving a course of treatment under a national health insurance system of an EEA State for this condition with any of the following drugs – Alpropstadil (Caverject). (Muse), (Viridal) Apomorphine Hydrochloride (Uprima)Moxislyte Hydrochloride (Erecnos) Sildenafilil (Viagra) Tadalafil (Cialis) Thymoxamine Hydrochloride (Erecnos);

C)A man who is not a national of an EEA state but who is the member of the family of such a national who has an enforceable Community right to be treated no less favourable than the national in the provision of medical treatment and has erectile dysfunction and was sbeing treated for that condition on 14th September 1998 with any of the following drugs - Alpropstadil (Caverject). (Muse), (Viridal) Apomorphine Hydrochloride (Uprima)Moxislyte Hydrochloride (Erecnos) Sildenafilil (Viagra) Tadalafil (Cialis) Thymoxamine Hydrochloride (Erecnos);for

Please contact the [medicines management](#) for further guidance where necessary.